

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY in Colorado

A CDPHE-licensed Class B (Non-Medical) Home Care Agency — private pay & Medicaid.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Colorado model — read this first

In Colorado, an agency that sends caregivers into people's homes to help with daily living — bathing, dressing, meals, light housekeeping, companionship — is licensed by the Colorado Department of Public Health and Environment (CDPHE) as a Home Care Agency. There are two classes: Class A (Skilled) for nursing/therapy, and Class B (Non-Medical) for personal care only. A non-medical agency holds the Class B license under C.R.S. Title 25, Article 27.5 and the rules at 6 CCR 1011-1 Chapter 26. Good news for your business plan: unlike some states, Colorado's Medicaid personal-care and homemaker services are mostly AGENCY-provided — so once you're licensed and enrolled, Medicaid is a real revenue stream alongside private pay. This guide walks the whole path, broken down so a first-timer can follow it.

Colorado is agency-friendly for Medicaid — a real advantage. Some states (like California) route most Medicaid personal care to individual providers the client hires, largely bypassing agencies. Colorado is different: personal care and homemaker services are primarily delivered by enrolled AGENCIES that employ the caregivers and bill Medicaid. As of July 1, 2025, these services moved into a new program called Community First Choice (CFC) — an entitlement with no waiting list. So your agency can build a genuine Medicaid book of business (Part 8), while private pay lets you start earning right away.

Accreditation — optional, and Colorado even rewards it. Colorado does NOT require national accreditation for a Class B license. In fact the rules give an accredited agency a 10% discount on its renewal fee. Accreditation (ACHC, CHAP, or The Joint Commission) is also often preferred by VA contracts, long-term-care insurers, and franchise brands. Every PolicyReady product is built to those standards behind the scenes; treat accreditation as a goal for year two (see Part 9).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened an agency — where we name a thing (an agency, a form, a rule, an insurance type), we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with CDPHE, HCPF, and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

Ten milestones take you from idea to a licensed, operating, paid agency. Each part below is one rung.

1	Decide this is your business	A licensed non-medical home-care agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, and the right insurance & bonding (Part 3)
3	Get the Class B license	CDPHE Letter of Intent, application, the fee (Part 4)
4	Set up your administrator	The 24-hour administrator training + a qualified supervisor (Part 5)

5	Recruit & screen caregivers	Competency, CBI/CAPS checks, the 12 training topics (Part 5)
6	Set up EVV & policies	Sandata EVV, P&P Manual, your first survey (Part 6)
7	Serve private-pay clients	Start earning while Medicaid enrollment is pending (Part 8)
8	Enroll for Medicaid (CFC)	Gainwell provider enrollment + the personal-care/homemaker specialty (Part 8)
9	Get your first clients	Discharge planners, Case Management Agencies, AAAs (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. Do your entity and Letter of Intent early, start the administrator training, and launch on private pay so you're generating revenue while the Medicaid enrollment (Gainwell + the required CO.Train training) runs in the background. Colorado also requires workers' compensation from your first employee.

PART 1

What This Business Is

In short: A CDPHE-licensed Class B (Non-Medical) Home Care Agency — caregivers who help clients with daily living in their own homes. Personal care, homemaker, and companion work under one license.

Colorado law defines a home care agency as an entity that provides services in a person's home. A Class B / Non-Medical agency provides personal care services ONLY — it may not provide any skilled healthcare service. Colorado's definition of “personal care services” is broad and already folds in homemaker and companion work: help with the activities of daily living (bathing, dressing, eating, transferring, mobility, toileting), plus housekeeping, personal laundry, medication reminders, and companionship.

Class B (non-medical) vs. Class A (skilled)

	Class B — Non-Medical	Class A — Skilled
What it is	Personal care / homemaker / companion	Nursing, therapy, wound care
Who delivers it	Trained caregivers	RNs, LVNs, PT/OT/ST
Supervision	A qualified supervisor (no RN required)	RN supervision required
Medications	Reminders/assistance only — no administration	Nurses administer/manage medications

This guide is for the Class B non-medical license. Skilled (Class A) is a separate, larger license you could add later. Note: Colorado does NOT license “homemaker” or “companion” as separate categories — they're all inside the single personal-care (Class B) license.

The clinical line to respect. A Class B agency provides personal care, not skilled care — caregivers can remind and assist a client with self-administered medication, but not administer medications or do nursing tasks. Keep your scope inside personal care and you stay inside the Class B license.

Key terms. CDPHE = Colorado Dept. of Public Health & Environment (your licensor). HCPF = Dept. of Health Care Policy & Financing (runs Medicaid). Class B = the non-medical home-care license. CFC = Community First Choice (the Medicaid program that now covers personal care/homemaker). EBD = Elderly, Blind & Disabled waiver. CDASS/IHSS = the consumer-directed options (client hires their own worker). CMA = Case Management Agency (assessment & referral hub). CBI = Colorado Bureau of Investigation (fingerprint checks). CAPS = Colorado Adult Protective Services check. EVV = Electronic Visit Verification (Sandata). NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. Colorado has one of the fastest-growing 65-plus populations in the country — the state's older-adult population is projected to keep climbing sharply through the 2030s — and it's an agency-friendly Medicaid state, so demand for in-home caregivers here comfortably outstrips supply. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: CDPHE licenses and surveys you, HCPF + Gainwell enroll you for Medicaid and the Case Management Agencies send you clients, the CBI and CAPS systems screen your people, and Sandata EVV verifies Medicaid visits.

CDPHE — Health Facilities & EMS Division (your licensor)

The Colorado Department of Public Health and Environment issues your Class B license, writes the 6 CCR 1011-1 Chapter 26 standards, reviews your application, and conducts the licensing inspection and ongoing surveys. Health Facilities licensure: 303-692-2836; cdphe.healthfacilities@state.co.us; cdphe.colorado.gov/home-care-agencies.

HCPF, Gainwell & the Case Management Agencies (Medicaid & referrals)

The Department of Health Care Policy & Financing (HCPF) runs Health First Colorado (Medicaid) and sets the personal-care/homemaker rates. You enroll as a provider through Gainwell (the fiscal agent's portal). Members access services through regional Case Management Agencies (CMAs) — since 2024 a single CMA per region assesses clients, builds care plans, and refers to enrolled providers. Statewide ADRC line 1-844-265-2372.

CBI & CAPS (background screening)

Colorado screens your people two ways: owners, managers, and the administrator get a fingerprint-based Colorado Bureau of Investigation (CBI) criminal-history check; and before hiring any direct-care worker you must run a CAPS check (the Colorado Adult Protective Services data system) plus a Colorado criminal-history check on the caregiver (Part 5).

EVV — Electronic Visit Verification (Sandata)

For Medicaid personal-care and homemaker visits, Colorado requires Electronic Visit Verification — an electronic record of who provided care, for whom, where, and when. Colorado's state EVV vendor is Sandata (or an approved “Provider Choice” system that interfaces with Sandata), and EVV compliance affects claim payment. Private-pay-only work isn't subject to EVV.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Colorado you form it online at the Secretary of State (coloradosos.gov, Business Division 303-894-2200) — Colorado is online-only, the LLC filing fee is about \$50, and it's one of the cheapest and fastest states in the country to file in. Every Colorado entity then files an annual Periodic Report (\$25, in a window around your formation-anniversary month; \$50 late fee), so calendar it. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

1. **Go to the Colorado Secretary of State.** File online at coloradosos.gov (Business Division, 303-894-2200) — Colorado is online-only.
2. **File Articles of Organization (LLC).** The filing fee is \$50. Name the LLC and list a registered agent (can be you).
3. **Adopt an Operating Agreement.** The internal document setting out ownership, management, and money.
4. **Calendar your Periodic Report.** Every Colorado entity files an annual Periodic Report — \$25 — in a window around your formation-anniversary month; late fee \$50.

No newspaper publication in Colorado. Forming the entity is a same-day online filing — Colorado doesn't require a formation notice or newspaper publication.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

1. **Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
2. **Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
3. **Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
4. **Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. Colorado Medicaid (Health First Colorado) uses it to identify your agency when you enroll and bill through Gainwell (Part 8). Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

1. **Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
2. **Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a major bank with lots of Colorado branches — Chase, Wells Fargo, U.S. Bank, or FirstBank (a large Colorado-based bank) — or a local credit union like Bellco or Ent. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home-care agency sends employees into clients' homes, so your risk is different from a facility's — and Colorado requires evidence of liability insurance for licensure:

Coverage	What it protects against	Must-have?
General & Professional Liability	Bodily injury/property damage + claims about the CARE	Yes — required for licensure
Abuse & Molestation	Allegations of abuse by a caregiver (GL excludes this!)	Yes — high exposure
Employee Dishonesty / Surety Bond	Theft from a client's home by an employee	Yes — clients & payers expect it
Non-Owned & Hired Auto	Caregivers driving their own cars for/with clients	Yes — critical for home care
Workers' Compensation	Caregiver injuries (lifting/transferring is high-risk)	Required in Colorado from your first employee
Cyber Liability	A breach of client health information (HIPAA)	Strongly recommended

Two coverages home-care owners forget — and one Colorado requires. Colorado requires workers' comp as soon as you have any employee, and you file evidence of liability insurance to get licensed. Add non-owned/hired auto (caregivers drive their own cars to clients) and a caregiver dishonesty bond (clients ask whether caregivers are “bonded”) — both are core home-care risks a basic policy leaves out.

Real places to get it

Buy through an independent agent/broker who writes home-care programs. Carriers/programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan, Gallagher, and NIP Group/HomeCareUSA. Find a local Colorado agent at [trustedchoice.com](https://www.trustedchoice.com) (filter home care), or ask a franchise or peer. Confirm the carrier writes non-medical home care in Colorado.

What to ask the insurance agent

“Do you write NON-MEDICAL home care — which carriers and what's the minimum premium?”

“Is professional liability included with GL, and is abuse & molestation included or a sub-limit?”

“Do you include non-owned & hired auto, and an employee-dishonesty bond?”

“What limits do Medicaid, the VA, or long-term-care insurers require to contract?”

“What will Colorado workers' comp cost given my caregiver payroll?”

PART 4

Get Your Class B License — Step by Step

In short: *File a Letter of Intent, submit the application with your insurance and background clearances, pass the licensing inspection, and receive your Class B (Non-Medical) Home Care Agency license.*

4.1 Before you apply

- ☐ Entity formed and in good standing; EIN and NPI obtained
- ☐ Liability insurance bound (and workers' comp); a dishonesty bond arranged
- ☐ A qualified administrator and supervisor identified (Part 5)
- ☐ Your policies & procedures written (your P&P Manual) and forms ready
- ☐ Owner/manager/administrator fingerprint (CBI) checks underway

4.2 The application

- 1. File a Letter of Intent (LOI).** Start on the CDPHE Health Facilities licensing page; the LOI opens your application.
- 2. Complete the license application.** Provide your ownership, the counties you'll serve, evidence of liability insurance, and your criminal-history clearances (Chapter 26 §5.2).
- 3. Pay the fee.** The Class B initial license fee is \$2,200 (renewal \$1,325) per the rule; confirm the current amount on CDPHE's July-1 fee schedule (or call 303-692-2836).
- 4. Pass the licensing inspection.** CDPHE's process includes an inspection/survey before (or shortly after) licensure — confirm the timing for your application.
- 5. Receive your Class B license.** Then keep it current and renew annually.

4.3 Timeline

CDPHE doesn't publish a fixed turnaround, so assemble a complete application (missing insurance proof or background clearances are the common delays) and start your Medicaid enrollment and caregiver hiring in parallel. Launch on private pay as soon as the license issues — you don't have to wait for Medicaid to start serving clients.

PART 5

The Administrator, Supervisor & Caregivers — by Position

In short: Colorado is specific about your administrator's qualifications and training and about background screening. Here it is, one role at a time — and a non-medical agency does NOT need an RN.

5.1 Your administrator / agency director (Chapter 26 §6.4)

Every agency must have a qualified administrator (who may be you). Here's what it takes:

- 1. Meet the qualifications.** At least 21 years old; a licensed physician/RN/health professional OR education and experience in health-service administration; with at least 2 years of healthcare or health-service-administration experience, including at least 1 year of supervisory experience in home care or a closely related program.
- 2. Complete the initial training.** 24 clock hours of administrator training within the first 12 months — 8 of them within the first month.
- 3. Keep up continuing education.** 12 clock hours each subsequent year.

You don't need to be a nurse. A Class B administrator can qualify through health-service-administration education and experience — you don't have to be an RN. Line up the 24-hour administrator training before or right after you apply.

5.2 Your supervisor (no RN required for Class B)

Because a Class B agency provides only personal care, you do NOT need a registered nurse. The rule requires a qualified supervisor to be available during all operating hours (§5.2(E)). (RN supervision attaches only to skilled/Class A services.) Your supervisor sets up each client's care, oversees caregivers, and enforces your supervision policy.

5.3 Your caregivers — competency & training

- **Competency, not a fixed hour count:** before working independently, each caregiver passes a competency evaluation — an observed skills check plus written testing overseen by a qualified person (§5.9(B)). Colorado's rule is competency- and topic-based rather than a set number of pre-service hours; build a solid orientation and skills check-off anyway.
- **Ongoing training — at least 12 topics per 12 months:** for all direct-care staff, covering consumer rights and abuse prevention/reporting, behavior management, disaster/emergency procedures, infection control, and more (§6.6(D)).
- **TB testing:** not mandated by Chapter 26 — CDPHE recommends but doesn't require TB screening for private providers. Many agencies screen anyway as a best practice; confirm current guidance.

5.4 Background checks — the system you must run

- 1. Fingerprint (CBI) your owners, managers & administrator.** A fingerprint-based Colorado Bureau of Investigation (state + national) criminal-history check is required for owners, managers, and the administrator — submitted within 10 days of any change.
- 2. Run a caregiver criminal-history check.** For direct-care staff, a Colorado criminal-history records check within 90 days before employment (the rule requires this name-based check for line caregivers — not a fingerprint check).
- 3. Run a CAPS check before hiring any direct-care worker.** Query the Colorado Adult Protective Services (CAPS) data system to screen against substantiated findings of abuse, neglect, or exploitation of at-risk adults (mandatory since 2019). CAPS Check Unit: ccu.colorado.gov.
- 4. Document everything.** Keep the dated results in each employee file — a surveyor will ask to see them.

5.5 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

EVV, Policies & Getting Operational

In short: Write your policies, set up Sandata EVV for Medicaid visits, and be ready for CDPHE's survey. EVV is a hard gate to Medicaid payment, so set it up before you take a Medicaid client.

6.1 Your policies & procedures (the P&P Manual)

CDPHE expects written policies covering consumer rights, intake and assessment, the care plan and supervision, caregiver competency and training, complaint handling, incident and abuse/neglect reporting, emergency preparedness, and infection control. This is your operating backbone and the first thing a surveyor reads — a complete P&P Manual and matching forms is the single biggest time-saver at survey.

6.2 Set up EVV (before your first Medicaid visit)

Electronic Visit Verification is mandatory for Medicaid personal-care and homemaker visits and is the gate to getting paid.

1. **Pick your EVV system.** Use the free state Sandata system, or a HCPF-approved “Provider Choice” third-party system that interfaces with Sandata.
2. **Onboard and train.** Complete EVV onboarding and set up your caregivers, clients, and authorizations.
3. **Capture every visit.** Each visit records who/what/where/when; complete visit maintenance to fix any exceptions.
4. **Bill against verified visits.** EVV compliance affects claim payment — a visit that isn't verified creates payment problems.

Private pay is EVV-free. EVV applies to Medicaid visits. If you launch on private pay and long-term-care insurance, you don't need EVV until your first Medicaid client — one more reason private pay is the easier on-ramp.

6.3 Complete your Medicaid provider training

Before you can enroll to bill Medicaid (Part 8), Colorado requires HCBS/CFC provider training on the state's CO.Train platform (you must pass with at least 80%). Do this while your license application is pending so you're ready to enroll the moment you're licensed.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (privately or by a Case Management Agency), you assess and set up the care plan, a caregiver delivers the hours (with EVV for Medicaid), a supervisor oversees, and you review and adjust. Here is the whole journey, stage by stage.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, or a Case Management Agency case manager (Part 10). Someone asks whether you can staff care for a specific person.

What you do:

- Take the basics (who needs care, what tasks, how many hours, where, when to start, and who pays — private, Medicaid/CFC, VA, or long-term-care insurance). Confirm the need is non-medical.

Stage 2 — Assessment & the care plan

Do an in-home assessment and build a care plan: the specific tasks (bathing, dressing, meals, housekeeping, transport, companionship), the schedule and hours, and any safety concerns. For Medicaid clients, the CMA's assessment and authorization set the approved units and services; your plan aligns to it.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement covering services, schedule, rate/payer, client rights, and how service can end. The supervisor sets up care, and you match a screened, competent caregiver to the client.

Stage 4 — Delivering the hours (with EVV for Medicaid)

The caregiver delivers the scheduled care; for Medicaid visits, each one is captured in EVV. Keep visit notes and protect the client-caregiver match — consistency of caregiver is the number-one driver of client satisfaction and retention.

Stage 5 — Supervision & quality

Supervise per your policy: check in on the schedule you set with the family, monitor the caregiver's performance and the client's changing needs, and document it. Run your quality program — satisfaction, complaints, incident reports — and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes & reassessment

As the client's needs change, update the care plan; for Medicaid clients, work with the CMA on reassessments and new authorizations (units can go up or down). Keep authorizations current — delivering unauthorized units doesn't get paid.

Stage 7 — Discharge & transition

When care ends — recovery, a move to a higher level of care, or a hospice transition — follow your discharge policy, give proper notice, coordinate with the family and any CMA, and document the transition. A good discharge and a follow-up call turn families into referral sources.

PART 8

How You Get Paid — Private Pay, Medicaid (CFC), VA & LTC Insurance

In short: Start on private pay and long-term-care insurance the day you're licensed; add Medicaid (now through Community First Choice) once you enroll. Colorado personal care/homemaker is billed per 15-minute unit.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start immediately on your license; highest rate; no EVV/Medicaid enrollment
Medicaid (Community First Choice)	Health First Colorado personal care & homemaker	A real agency market in Colorado; requires enrollment & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network, or A&A-funded private pay

8.1 Private pay — your launch market

The day your license issues, you can serve private-pay clients — families paying out of pocket, typically billed by the hour with a visit minimum (often 3–4 hours). Colorado runs above the national median (national non-medical care is roughly \$34–\$35/hour; Colorado tends higher — verify the current Colorado figure with a CareScout/Genworth cost-of-care report). You set your own rate above your fully-loaded caregiver cost. Long-term-care insurance is part of this stream: many clients have an LTC policy that reimburses in-home personal care once ADL/cognitive benefit triggers are met — you bill the family or the insurer.

8.2 Medicaid — Community First Choice (the agency market)

As of July 1, 2025, Colorado's Medicaid personal-care and homemaker services moved into Community First Choice (CFC) — a state-plan entitlement with no waiting list — and these services are primarily delivered by enrolled agencies. (The Elderly, Blind & Disabled waiver still exists for other services; the consumer-directed options — IHSS and CDASS — let a client hire their own worker instead of an agency, so know they exist but they're not your channel.)

How to get set up to bill Medicaid

1. **Hold your Class B license first (Part 4).**
2. **Complete the CO.Train HCBS/CFC provider training** (pass at least 80%) — Part 6.3.
3. **Enroll through Gainwell.** Apply in the Provider Web Portal as Provider Type 36 (HCBS), with the Personal Care/Homemaker specialty (SP 666) — attach your training certificate, W-9, and banking info; a separate application/fee applies per location; revalidate every 5 years.
4. **Set up EVV (Part 6)** before you deliver a Medicaid visit.

8.3 The rate & billing

Colorado pays personal care and homemaker per 15-minute unit (not per hour). On the schedule effective October 1, 2025:

Service	Code	Per 15-min unit	≈ Hourly
Personal Care	T1019	\$6.96	~\$27.84
Homemaker	S5130	\$6.49	~\$25.96

An enhanced (“HX”) rate is a little higher. Pull the current figures from HCPF's rate schedule (and the CFC fee schedule) before you rely on them. Bill through Gainwell against verified EVV visits, and confirm the current timely-filing window (Colorado Medicaid generally allows a generous filing period — verify).

8.4 Veterans

- **VA Homemaker / Home Health Aide (H/HHA):** the VA's non-medical in-home program — a VA nurse assesses, then a VA-approved agency provides the aide. This is your entry point to serve veterans; contract through the VA Community Care Network (Colorado is currently in the TriWest-administered region, though the VA is reorganizing the network — verify the current administrator).
- **Veteran-Directed Care:** the veteran gets a flexible budget to arrange their own services — you can be their agency.
- **Aid & Attendance:** an increased VA pension (2025–2026: up to about \$2,424/month for a single veteran, \$1,558 for a surviving spouse — verify on VA.gov) that a veteran can apply toward your private-pay care.

PART 9

What It Costs & How Long It Takes

In short: Budget the license, the administrator training, insurance & bonding, EVV, payroll float, and (optionally) accreditation. Plan to launch on private pay, then add Medicaid.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
Class B license	Initial CDPHE non-medical home-care license	\$2,200 (renewal \$1,325)
Administrator training	24-hr initial course + 12-hr annual CE	Several hundred \$
Business setup	SOS \$50 + \$25/yr periodic report, EIN (free), NPI (free), legal	~\$75 + legal
Insurance & bond	Liability (required) + abuse + non-owned auto + dishonesty bond + workers' comp	Get quotes — annual
EVV	Sandata (free) or an approved Provider Choice system	\$0–modest
Software	Scheduling / EVV / billing / payroll platform	Monthly SaaS
Payroll float	You pay caregivers before Medicaid/insurers pay you	Working capital — plan for it
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional)	ACHC / CHAP / Joint Commission — earns a 10% renewal discount	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. Home care is payroll-heavy: you pay caregivers weekly, but Medicaid and insurers pay you weeks later. Private-pay clients (billed in advance or on short terms) smooth this out. Line up a business line of credit before you scale Medicaid units.

9.2 If you pursue accreditation

- **Why:** some VA contracts, long-term-care insurers, and franchise brands prefer or require accreditation — and Colorado gives accredited agencies a 10% discount on the renewal fee.
- **Who:** ACHC, CHAP, and The Joint Commission all accredit home care; expect roughly \$7,000–\$14,500 over a 3-year cycle plus staff time.
- **When:** most non-medical agencies launch on the state license and add accreditation in year two.

9.3 Timeline

Early	Entity, training, policies	Form the LLC, start the 24-hr admin training + CO.Train, write your P&P
Weeks	Insurance & background	Bind coverage & bond; run CBI/CAPS checks
Submit	CDPHE application	LOI + application + \$2,200; pass the inspection
Right away	Start private pay	Serve private-pay & LTC-insurance clients on your license
Parallel	Enroll for Medicaid	Gainwell (Type 36 / SP 666) + Sandata EVV setup
Ongoing	Referrals & full census	Case Management Agencies + discharge planners + AAAs

9.4 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$35/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, home-health/hospice partners, the Case Management Agencies, the Area Agencies on Aging and ADRC, care managers, and senior communities. Build these relationships before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the highest-volume source for post-hospital home support — introduce yourself, leave a one-pager, and make same-day starts easy.
- **Case Management Agencies (CMAs):** for Medicaid clients, the regional CMA assesses members and refers to enrolled providers (there are 15 CMAs across 20 service areas since the 2024 redesign) — once enrolled, stay in close contact with the CMA covering your area.
- **Skilled home-health & hospice partners:** they refer out the non-medical/companion hours they don't provide; build reciprocal partnerships.
- **Area Agencies on Aging & the ADRC:** Colorado has 16 AAAs and the statewide Aging & Disability Resources for Colorado line (1-844-265-2372) that field families looking for in-home help — get listed and known.
- **Geriatric care managers & senior living communities:** Aging Life Care professionals place clients with vetted agencies, and assisted-/independent-living communities refer supplemental one-on-one care.
- **2-1-1 Colorado:** list your services with 2-1-1 (dial 211; 866-760-6489) — a free statewide referral channel.

10.2 What to ask each referral source

"How do you decide which home-care agency to refer a family to?"

"What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?"

"Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?"

"How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?"

"What makes an agency easy to work with, and what gets one dropped from your list?"

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the administrator & staff

Who	Contact	What to verify / ask
CDPHE — Health Facilities & EMS	303-692-2836 · cdphe.colorado.gov/home-care-agencies	The LOI & application; the current Class B fee; the inspection; renewal
CO.Train (HCBS/CFC provider training)	hcpf.colorado.gov (HCBS provider enrollment)	The required provider training (pass ≥80%) before Medicaid enrollment
Colorado Bureau of Investigation (CBI)	cbi.colorado.gov	Fingerprint checks for owners/managers/administrator
CAPS Check Unit (CDHS)	ccu.colorado.gov · cdhs_ccu@state.co.us	The required CAPS check before hiring direct-care staff
City/County business licensing	your jurisdiction	Any local business license/permit for your office

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Colorado Secretary of State	303-894-2200 · coloradosos.gov	Forming your LLC (\$50); the \$25 periodic report
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	npes.cms.hhs.gov	Your Type 2 NPI and the right taxonomy
Insurance agent (home-care program)	trustedchoice.com ; carriers in Part 3.4	Liability + abuse + non-owned auto + dishonesty bond + workers' comp
Colorado Div. of Workers' Comp	cdle.colorado.gov (DOWC)	Setting up your required workers'-comp policy

Funding, EVV & referrals

Who	Contact	What to verify / ask
HCPF / Gainwell — provider enrollment	hcpf.colorado.gov/hcbs-provider-enrollment-information	Enrolling as Type 36 / SP 666; the CFC structure; revalidation
HCPF — rates	hcpf.colorado.gov (provider rates)	The current personal-care (T1019) & homemaker (\$5130) unit rates
HCPF — EVV / Sandata	hcpf.colorado.gov (EVV)	EVV onboarding, Provider Choice option, and compliance
Your Case Management Agency	1-844-265-2372 (ADRC) · HCPF CMA directory	Which CMA covers my area; how referrals & authorizations work
VA Community Care / TriWest	va.gov/communitycare	The VA Homemaker/HHA program; the current CCN administrator

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging (16 in CO)	cdhs.colorado.gov (State Unit on Aging) · c4a.info	Local information & referral for older adults
Aging & Disability Resources (ADRC)	1-844-265-2372	Statewide options counseling & referral
2-1-1 Colorado	dial 211 · 866-760-6489 · 211colorado.org	Free statewide referral line — list your services
Accreditor (optional) — ACHC/CHAP/TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; the 10% renewal discount; payer requirements

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

You can do this

Opening a Colorado non-medical home care agency is one of the more accessible care businesses to start — no building, no nurse-owner, and Colorado is genuinely agency-friendly for Medicaid. Take it one rung at a time: form the business, get the Class B license, train your administrator, recruit and screen great caregivers, launch on private pay and long-term-care insurance, add Medicaid through Gainwell and the Community First Choice program, and build referral relationships with discharge planners, the Case Management Agencies, and the aging network. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in C.R.S. Title 25 Art. 27.5, 6 CCR 1011-1 Chapter 26, and current HCPF/Community First Choice information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with CDPHE, HCPF, and qualified counsel before you rely on it. Confirm the current license fee, the personal-care/homemaker unit rates, and your EVV setup before you rely on them.